
Attachment 1. Protocol JPBL Study Schedule

Study Schedule, Protocol I3Y-MC-JPBL

Perform procedure as indicated.

			Baseline		Patients on Study Treatment				Postdiscontinuation Follow-Up	
			Cycle		1	2 – 3	4 and Beyond (if Applicable)		Short-Term Follow-Up ^a	Long-Term Follow-Up ^a
			Visit		1	2 – 3	4 and Beyond (if Applicable)		801	802 - 8XX
			Approximate Duration (days)		28	28	28	28	30	Variable
			Relative day within a cycle		≤28	≤14	1	15±3	1 ^p	1 ^p
Procedure Category	Procedure	Protocol Reference								
Study Entry /Enrollment	Informed Consent Form signed ^q	Section 8.1	X ^q							
	Inclusion/Exclusion evaluation	Section 7		X						
Medical History	Medical History			X						
	Historical illnesses	Section 12.2.3		X						
	Habits assessment	Section 12.2.3		X						
Physical Examination	Height	Section 12.2.3		X						
	Weight	Section 12.2.3		X	X	X	X	X	X	
	Vital Signs (Temp, BP, HR, RR)	Section 12.2.3		X	X	X	X	X	X	
	ECOG performance status	Section 7.1 Attachment 4		X	X	X	X	X	X	
Tumor Assessment	Tumor measurement (palpable or visible) ^c	Section 10.1.1	X ^c				X ^c	X ^c	X ^c	X ^c
	Radiologic imaging according to RECIST ^b	Section 10.1.1 Attachment 5	X ^b				X ^b	X ^b	X ^b	X ^b
	Bone Scintigraphy ⁱ	Section 10.1.1 Attachment 5	X ⁱ					X ⁱ	X ⁱ	X ⁱ
	X-ray or CT scan with bone windows or MRI ^j	Section 10.1.1 Attachment 5	X ^j				X ^j	X ^j	X ^j	X ^j

			Baseline		Patients on Study Treatment				Postdiscontinuation Follow-Up	
		Cycle	BL		1	2 – 3	4 and Beyond (if Applicable)		Short-Term Follow-Up ^a	Long-Term Follow-Up ^a
		Visit	0		1	2 – 3	4 and Beyond (if Applicable)		801	802 - 8XX
		Approximate Duration (days)	28		28	28	28		30	Variable
		Relative day within a cycle	≤28	≤14	1	15±3	1 ^p	1 ^p		
Procedure Category	Procedure	Protocol Reference								
Survival Information ^d		Section 10.1							X ^d	X ^d
Adverse Event Collection/CTCAE Grading ^f		Section 10.3		X ^f	X ^f		X ^f		X ^f	X ^f
Concomitant Medications (with analgesics)		Section 9.6		X	X		X		X	
Lab/ Diagnostic Tests	Central hematology	Attachment 2		X	X	X	X		X	
	Central serum chemistry	Attachment 2		X	X	X	X		X	
	Local FSH and estradiol levels ⁿ	Attachment 2		X ⁿ						
	Local serum pregnancy test ^o	Attachment 2		X ^o						
	Central pharmacokinetic (PK) sampling ^m	Attachment 7			X ^m	X ^m	X ^m			
	Pharmacogenetic blood sample	Section 10.4.2.2			X					
	Biomarker plasma sample	Section 10.4.2.3			X					
	Local ECG ^e	Section 10.3.2.1		X ^e	X ^e	X ^e			X ^e	
	Archived Tumor Sample ^h	Section 10.4.2.1			X ^h					

			Baseline		Patients on Study Treatment			Postdiscontinuation Follow-Up	
			BL		1	2 – 3	4 and Beyond (if Applicable)	Short-Term Follow-Up ^a	Long-Term Follow-Up ^a
			0		1	2 – 3	4 and Beyond (if Applicable)	801	802 - 8XX
			28		28	28	28	30	Variable
			≤28	≤14	1	15±3	1 ^p	1 ^p	
Procedure Category	Procedure	Protocol Reference							
Study Drug	Fulvestrant Therapy ^g	Section 9.1			Days 1 and 15 of Cycle 1, then Day 1 of Cycle 2 and beyond ^g				
	LY2835219 Therapy ^g	Section 9.1			Every 12 hours on Days 1 through 28 of every cycle ^g				
Health Outcomes	BPI, EORTC QLQ-C30, EORTC BR23, EQ-5D 5L ^k	Section 12.2.11		X ^k		X ^k	X ^k	X ^k	
	Skeletal-Related Events assessment ^l	Section 12.2.11		X ^l		X ^l	X ^l	X ^l	X ^l
	Hospitalization	Section 12.2.11.4		X		X	X	X	
	Transfusion	Section 12.2.11.4		X		X	X	X	

Abbreviations: BL = baseline; Temp = Temperature; BP = Blood Pressure; CT = computed tomography; CTCAE = Common Terminology Criteria for Adverse Events; ECG = electrocardiogram; ECOG = Eastern Cooperative Oncology Group; GnRH = gonadotropin-releasing hormone; HR = heart rate; IV = intravenous; PK = pharmacokinetics; MRI = magnetic resonance imaging; RECIST = Response Evaluation Criteria in Solid Tumors; RR = respiratory rate; SAEs = serious adverse events; FSH = follicular stimulating hormone.

- ^a Short-term follow-up begins the day after the patient and the investigator agree that the patient will no longer continue study treatment and lasts approximately 30 days; the associated study procedures are performed once at the end of this period. Long-term follow-up begins the day after short-term follow-up is completed and continues until the patient's death or overall study completion; the associated study procedures are performed approximately every 12 weeks (± 14 days) for the duration of this period.

Study Schedule, Protocol I3Y-MC-JPBL (continued)

- b For patients with inoperable locally advanced breast cancer, MRI scan of the breast is performed locally at baseline (Day -28 to Day -1), between Day 1 and Day 7 of every second cycle beginning with Cycle 3 and continuing through Cycle 13, between Day 1 and Day 7 of every third cycle after Cycle 13, and within 14 days of clinical progression. For all patients, CT or MRI scan of the chest, abdomen, and pelvis is performed locally at baseline (Day -28 to Day -1), between Day 1 and Day 7 of every second cycle beginning with Cycle 3 and continuing through Cycle 13, between Day 1 and Day 7 of every third cycle after Cycle 13, and within 14 days of clinical progression. It is recommended that CT imaging of the abdomen and pelvis be performed with IV contrast whenever possible. If this is not feasible due to hypersensitivity or other conditions, then gadolinium-enhanced MRI is preferred. For patients with known serious allergic reactions to CT contrast material, a CT of the chest without contrast and gadolinium-enhanced MRI of the abdomen/pelvis are encouraged. For patients who discontinue study treatment without objectively measured progressive disease (PD), continue to evaluate tumor response approximately every 8 weeks for the first 12 months following randomization and thereafter approximately every 12 weeks by the same method used at baseline and throughout the study until the patient has objective disease progression or until study completion. After the patient has objective disease progression, radiologic tests are no longer required and the patient will continue with post-discontinuation follow-up until the patient's death or overall study completion.
- c Visible tumor (such as skin lesions) should be documented by photography and each photographic image of the tumor should include a ruler. For patients who discontinue study treatment without objectively measured progressive disease (PD), continue to evaluate tumor response approximately every 8 weeks for the first 12 months following randomization and thereafter approximately every 12 weeks by the same method used at baseline and throughout the study until the patient has objective disease progression or until study completion. After the patient has objective disease progression, tumor assessments are no longer required and the patient will continue with post-discontinuation follow-up until the patient's death or overall study completion.
- d Survival information is collected at baseline and during both study treatment and postdiscontinuation follow-up. During Long-Term Follow-Up, survival information is collected approximately every 12 weeks for the duration of this period. Although preferable to collect during a clinic visit, survival information may be collected by contacting the patient or family directly (for example, via telephone). Long-Term Follow-up data collection may include anticancer therapies.
- e A local ECG (no replicates required) should be obtained at baseline (Day -14 to Day -1), 2 to 4 hours after the LY dose on Cycle 1 Day 1, upon arrival at site but prior to fulvestrant dose on Cycle 1 Day 15, 2 to 4 hours after the LY dose on Cycle 4 Day 1, and at the short-term follow-up visit.
- f Data on SAEs that occur before the end of trial will be stored in the collection database and the Lilly Safety System. During Long-Term Follow-Up, only SAEs that are related to study drugs or protocol procedures will be collected. All adverse events possibly related to study drugs or protocol procedures should be followed until they resolve, are no longer considered to be possibly related, become stable or return to baseline, the patient starts a new therapy, the patient expires, or the patient becomes lost to follow-up. The frequency of evaluation is determined according to the judgment of the investigator.
- g LY2835219 should be administered orally every 12 hours on Days 1 through 28 of each cycle; patients should not consume food beginning 1 hour before and ending 1 hour after taking LY2835219. Fulvestrant 500 mg should be administered intramuscularly into the buttocks slowly (1 to 2 minutes per injection) as two 250-mg injections, one in each buttock; however, for patients with moderate hepatic impairment (defined as Child-Pugh Class B), including any patient who develops moderate hepatic impairment during study treatment, fulvestrant 250 mg should be administered intramuscularly into the buttock slowly (1 to 2 minutes) as one 250-mg injection.
- h Formalin-fixed paraffin-embedded tumor tissue (either block or 15-20 unstained slides) should be requested at the time of randomization. However, if this sample is not available for a patient, it will not constitute a protocol deviation

Study Schedule, Protocol I3Y-MC-JPBL (concluded)

- i Bone scintigraphy is performed locally at baseline (Day -28 to Day -1) for all patients. If available, prior bone scintigraphy (obtained as part of routine clinical care) within 45 days before Day 1 of Cycle 1 is also acceptable. Bone scintigraphy should be repeated for all patients between Day 1 and Day 7 of every sixth cycle beginning with Cycle 7, when complete response is identified in target disease, or when progression in bone is suspected. Importantly, RECIST v1.1 emphasizes that bone scintigraphy is not adequate to measure bone lesions; however, bone scintigraphy can be used to confirm the presence or disappearance of bone lesions. For patients who discontinue study treatment without objectively measured progressive disease (PD), continue to evaluate tumor response approximately every 6 months until the patient has objective disease progression or until study completion. After the patient has objective disease progression, bone scintigraphy is no longer required and the patient will continue with post-discontinuation follow-up until the patient's death or overall study completion.
- j One or more of these studies [X-ray, CT scan with bone windows, or MRI] is performed locally at baseline (Day -28 to Day -1), between Day 1 and Day 7 of every second cycle beginning with Cycle 3 and continuing through Cycle 13, between Day 1 and Day 7 of every third cycle after Cycle 13, and within 14 days of clinical progression only for patients with bone lesions identified by bone scintigraphy at baseline. For patients with new lesions identified by post-baseline bone scintigraphy, targeted assessment by X-ray, CT scan with bone windows, or MRI will be performed to confirm findings. For patients who discontinue study treatment without objectively measured progressive disease (PD), continue to evaluate tumor response approximately every 8 weeks for the first 12 months following randomization and thereafter approximately every 12 weeks by the same method used at baseline and throughout the study until the patient has objective disease progression or until study completion. After the patient has objective disease progression, radiologic tests are no longer required and the patient will continue with post-discontinuation follow-up until the patient's death or overall study completion.
- k mBPI-sf, EORTC QLQ-C30, EORTC QLQ-BR23 and EQ-5D 5L should be administered at baseline (Day -14 to Day -1), Cycle 2 Day 1, and then on Day 1 of every second cycle beginning with Cycle 3 and continuing through Cycle 13, on Day 1 of every third cycle after Cycle 13, and at Short-Term Follow-Up. Patients should complete these assessments before extensive interaction with site staff.
- l Skeletal-Related Events include pathological fracture, spinal cord compression, radiation to bone, and surgery to bone.
- m See Pharmacokinetic Sampling Schedule ([Attachment 7](#)).
- n FSH and estradiol levels are required only for women age <60 years and amenorrheic for at least 12 months.
- o A serum pregnancy test is required only for patients receiving ovarian suppression with a GnRH agonist.
- p For Cycle 2 and beyond, the start of a cycle may be delayed up to 7 days for logistical reasons and up to 14 days to allow sufficient time for recovery from toxicity possibly related to a study drug. Refer to Section [9.4.1.1.2](#).
- q Informed Consent Form is signed within 28 days prior to randomization of study drug and prior to performance of any protocol-specific tests/procedures.

Study Schedule for the extension period only, Protocol I3Y-MC-JPBL

Perform procedure as indicated.

		Patients on Study Treatment		Extension Period Follow-Up
Cycle		X-Y		Follow-Up ^a
Visit		501-5XX		901
Duration (days)		28		30
Relative day within a cycle		1	15	
Procedure Category	Procedure	Protocol Reference		
Adverse Events Collection/CTCAE Grading^b		Section 10.3	X	X
Study Drug	Fulvestrant Therapy ^c	Section 8.1.2	Days 1 and 15 of Cycle 1, then Day 1 of Cycle 2 and beyond ^c	
	LY2835219 Therapy ^c	Section 8.1.2	Daily every 12 hours ^c	

Abbreviations: CTCAE = Common Terminology Criteria for Adverse Events; PK = pharmacokinetics; SAEs = serious adverse events.

^a The extension period begins after study completion and ends at the end of trial.

^b Data on SAEs that occur before the end of trial will be stored in the collection database and the Lilly safety system.

^c LY2835219 should be administered every 12 hours on Days 1 through 28 of each cycle. Patients should not consume food beginning 1 hour before and ending hour after taking study drug.

Fulvestrant 500 mg should be administered intramuscularly into the buttocks slowly (1 to 2 minutes per injection) as two 250-mg injections, one in each buttock; however, for patients with moderate hepatic impairment (defined as Child-Pugh Class B), including any patient who develops moderate hepatic impairment during study treatment, fulvestrant 250 mg should be administered intramuscularly into the buttock slowly (1 to 2 minutes) as one 250-mg injection.